

MedQuest Diagnostic Laboratories — Clinical Chemistry

COMPREHENSIVE LABORATORY REPORT

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY — CONTAINS NO REAL PATIENT DATA

Patient: Howard J. Kimura	DOB: 11/28/1972	MRN: 2291847
Date: October 3, 2024 — Collected 07:42 AM (Fasting)		Provider: Dr. Rachel Bloom, MD — Internal Medicine

COMPREHENSIVE METABOLIC PANEL

Test	Result	Units	Reference	Flag
Sodium	139	mEq/L	136-145	
Potassium	4.1	mEq/L	3.5-5.1	
Chloride	101	mEq/L	98-107	
CO2 (Bicarb)	24	mEq/L	22-29	
BUN	22	mg/dL	7-25	
Creatinine	1.38	mg/dL	0.74-1.35	H
eGFR	58	mL/min/1.73m ²	>60	L
Glucose (Fasting)	187	mg/dL	70-99	H
Calcium	9.4	mg/dL	8.6-10.3	
Total Protein	7.1	g/dL	6.3-8.2	
Albumin	4.0	g/dL	3.5-5.0	
AST	28	U/L	10-40	
ALT	34	U/L	7-56	
Alk Phosphatase	82	U/L	44-147	
Total Bilirubin	0.7	mg/dL	0.1-1.2	

COMPLETE BLOOD COUNT

Test	Result	Units	Reference	Flag
WBC	7.8	K/uL	4.5-11.0	
RBC	4.2	M/uL	4.7-6.1	L
Hemoglobin	11.2	g/dL	13.5-17.5	L
Hematocrit	34.1	%	41-53	L
MCV	81	fL	80-100	
Platelets	224	K/uL	150-400	
Neutrophils %	62	%	50-70	
Lymphocytes %	28	%	20-40	

PHYSICIAN NOTES

Results reviewed by Dr. Rachel Bloom, MD on October 3, 2024.

Flagged values of clinical concern:

- Elevated fasting glucose (187 mg/dL): Consistent with poorly controlled type 2 diabetes. HbA1c pending. Medication adjustment warranted at next visit.
- Elevated creatinine (1.38) with mildly reduced eGFR (58): Suggests early stage CKD (Stage G3a), likely diabetic nephropathy. Urine albumin-creatinine ratio ordered. Nephrology referral considered if trend continues.
- Low hemoglobin/hematocrit: Mild normocytic anemia. Iron studies and reticulocyte count ordered to characterize further.

Patient contacted by nursing staff. Follow-up appointment scheduled in 4 weeks.

This document contains entirely synthetic, AI-generated data for demonstration purposes only. No real patient, provider, or institutional information is represented.